Disability Scare (QUE), Roland Morris Pain Scale, Back Pain Functional Scale (BPFS), and the PROMIS profile domains to assess function.

The patient's medical record should include but is not limited to:

- The assessment of the patient by the performing provider as it relates to the complaint of the patient for that visit,
- Relevant medical history (including a history of chronic vertebrogenic LBP for at least 6 months and failure of non-surgical management of at least 6 months),
- Results of pertinent tests/procedures (including pertinent radiological studies, which must include MRI images and radiology reports of evidence of Modic changes Type 1 or 2 in the L3-S1 vertebral body endplates),
- Signed and dated office visit record/operative report.

Policy Guidelines

There are 2 types of Modic changes found on Magnetic Resonance Imaging (MRI):
Type 1 – Vascular development in the vertebral body, inflammation and edema, vertebral endplate changes, vascularized fibrous tissues within the adjacent marrow, hypointensive signals.
Type 2 – Changes in the vertebral body's bone marrow including replacement of normal bone marrow by fat, and hyperintensive signals.

American Society of Interventional Pain Physicians (ASIPP)
(ASIPP) ASIIP clinical practice guidelines reviewed the evidence for facet joint interventions for managing chronic spinal pain. The guidelines make the following recommendations:

• The level of evidence is II with moderate strength of recommendation for cervical and lumbar RFA.
• The level of evidence is III with weak to moderate strength of recommendation with emerging evidence for thoracic RFA.
• For facet joint nerve ablation, the suggested frequency would be 6 months or longer (maximum of 2 times per year) between each procedure, provided that 50% or greater relief is obtained for 5-6 months.
• If the interventional procedures are applied for different regions, they may be performed at intervals of no sooner than one week or preferably 2 weeks for most types of procedures if they are not allowed to be performed in one setting or contraindicated.
• The therapeutic frequency for medial branch neurotomy should remain at intervals of at least 6 months per region with multiple regions involved.
It is further suggested that all regions be treated at the same time, provided all procedures are performed safely.
• In the treatment or therapeutic phase, the interventional procedures should be repeated only as necessary according to the medical necessity criteria.